

1. Module 1: Overview Medicare Program Basics
2. Navigation Instructions
3. Terms and Conditions

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4. Learning Objectives

After reviewing "Module 1: Medicare Program Basics" you will be able to explain:

- The different ways to get Medicare benefits
- Eligibility for Part A and Part B
- What is covered under Part A and Part B
- Original Medicare premiums
- Help for beneficiaries with limited income
- Original Medicare beneficiary protections
- Combining Original Medicare and Part D
- Medigap coverage

5. Training Roadmap: Module I

- Medicare Program Basics
- Original Medicare: Eligibility, Enrollment, Entitlement, and Premiums
- Help for Individuals with Limited Income
- Medicare Part A Benefits and Original Medicare Cost Sharing
- Original Medicare and Part D
- Original Medicare Beneficiary Protections
- Medigap Coverage

6. Title Page – Medicare Program Basics

7. Medicare Basics

- Medicare is the Federal health insurance program for individuals who are aged (65 and over) and younger individuals who are disabled.
- Medicare eligibility does not take into consideration an individual's income. However,
 - individuals may pay higher premiums based on income, and
 - low-income individuals may be eligible for additional assistance.
- Individuals can receive their Medicare medical coverage:
 - directly from the Federal Government, which pays for services on a fee-for-service basis (this program is known as "Original Medicare" or "Fee-for-Service Medicare"); or
 - through a private health plan.
- Individuals must receive their Medicare Part D outpatient drug benefits through a private health plan (even if they get their medical coverage through Original Medicare).

8. Medicare Background

- Medicare began as a fee-for-service program (now referred to as "Original Medicare") under which beneficiaries could receive their health care benefits from any Medicare provider and have the provider paid directly by the Federal government.
- Medicare evolved to give beneficiaries the option of receiving their benefits and care through managed care plans that maintained contracts with networks of providers.
- Beginning in 2006, Medicare began providing coverage for outpatient prescription drugs, such coverage is only available through private health plans.

9. Overview of Medicare Benefits and Coverage -- Parts A, B, C, and D

- Medicare coverage is often known by the part of Medicare law under which it is authorized or regulated
 - Part A is referred to as "Hospital Insurance Benefits." Part A also covers other inpatient care, including skilled nursing facilities, rehabilitation facilities, and hospice.
 - Part B is referred to as "Supplementary Medical Insurance Benefits." Part B covers a broad range of outpatient services such as physician care, and drugs that are administered by physicians or other health care professionals (such as vaccines and intravenous medications).
 - Part C regulates and authorizes Medicare Advantage plans, which must cover Part A (except for hospice) and Part B benefits.
 - Individuals enrolled in a Part C plan still get hospice benefits, but they are paid for by Original Medicare.
 - Part D covers prescription drug benefits (for self-administered drugs, such as those picked up at a pharmacy and taken at home) and regulates Medicare prescription drug plans.

10. Overview of Medicare – Part E

There is also a lesser-known Part E of Medicare law that regulates other miscellaneous programs including:

- Medicare cost plans (which also cover Part A and Part B benefits)
 - Medicare cost plans are only offered in a limited number of states and are most frequently found in rural areas.
- Medicare supplemental insurance (Medigap Plans)
- The program for all-inclusive care for the elderly (PACE)

11. Different Ways to Get Medicare

There are different ways that beneficiaries can choose to receive their Medicare coverage.

- Original Medicare (Part A and Part B coverage)
 - Can be combined with a Medicare Supplement Plan and/or a Medicare Prescription Drug Plan.
- Medicare Advantage Plans (Medicare Part C health plans, with or without Part D benefits)
- Medicare Prescription Drug Plans
- Medicare Cost Plans
- PACE Plans
- Medicare-Medicaid Plans

12. Different Ways to Get Medicare – Brief Overview

WAYS TO GET MEDICARE COVERAGE ¹				
COVERAGE TYPE	BENEFITS			
	Part A and B benefits	Some of the cost sharing for Part A and B Benefits	Part D Benefits	Other benefits
Original Medicare ³	X			
Part C (Medicare Advantage) ⁴	X	X	May cover depending on plan	X (most offer additional benefits)
Cost Plans ^{2,3}	X	X	May cover depending on plan	X (most offer additional benefits)
Medicare Prescription Drug Plans (PDPs)			X	
Original Medicare with a supplemental plan (Medigap) ³	X (Original Medicare)	X (Medigap)		Some Medigap Plans cover foreign travel emergencies
PACE plans ²	X	X	X	X (Adult day center, some meals, other benefits)
Medicare-Medicaid Plans ²	X	X	X	X Integrates Medicaid benefits with Medicare benefits

¹ Brief overview. As detailed later, some ways can be combined and not all beneficiaries are eligible for all types of coverage.

² These types of plans are generally limited to certain geographic areas and are not available everywhere.

³ These plans can be combined with coverage under a PDP.

⁴ As discussed later, a few MA plan types (PFFS and MSAs) can be combined with coverage under a PDP.

13. Title Page – Original Medicare: Eligibility, Enrollment, Entitlement, and Premiums

14. Original Medicare

- Original Medicare covers only Part A and Part B benefits
 - Part A benefits include inpatient hospital, skilled nursing facility, hospice, and home health services
 - Part B benefits include outpatient and professional services such as those provided by a doctor or non-physician professional, clinical lab services, durable medical equipment, preventive services, and other outpatient medical services

15. Eligibility for Part A and Part B

To be eligible for Medicare Part A and Part B, an individual must:

(1) Be age 65 or older, or be under age 65 with certain disabilities, including:

- all who get disability benefits from Social Security or certain disability benefits from the Railroad Retirement Board for 24 months.
- individuals with Amyotrophic Lateral Sclerosis (ALS), often referred to as Lou Gehrig's Disease or have an end-stage renal disease (ESRD).

(2) Be a U.S. resident; and

- be either a U.S. citizen, or
- be an alien who has been lawfully admitted for permanent residence and has been residing in the United States for 5 continuous years before the month of applying for Medicare.

16. Medicare Enrollment – Parts A and B

Some people are automatically enrolled in Parts A and B:

- Subject to the Part B exception below for Puerto Rico:
 - Individuals who are already getting benefits from Social Security or the Railroad Retirement Board (RRB) will automatically be enrolled in Part A and Part B starting the first day of the month they turn 65. (If their birthday is on the first day of the month, Part A and Part B will start the first day of the prior month.) These individuals are also allowed to refuse Part B coverage. (See Medicare Part B for the potential consequences of refusing Part B).
 - Individuals with disabilities who are under age 65 are automatically enrolled in Parts A and B the month after they have received Social Security or Railroad Retirement disability benefits for 24 months. However, they have an opportunity to refuse Part B coverage.
 - Individuals with ALS (Amyotrophic Lateral Sclerosis, also called Lou Gehrig's disease) get Part A and Part B automatically the month their Social Security disability benefits begin.
- Individuals living in Puerto Rico are not automatically enrolled in Part B. They must sign up for it.

17. Medicare Enrollment – Parts A and B, continued

Other individuals will have to sign up if they want to be enrolled in Parts A and/or B.

- Individuals who are close to 65 but are not getting benefits from Social Security or the Railroad Retirement Board (RRB) may sign up for Parts A and B during their **initial enrollment period**, which begins 3 months before their 65th birthday, including the month they turn 65 and ends 3 months after. (See Medicare Part B for the potential consequences of failing to sign up for Part B when first eligible).
- Individuals with end-stage renal disease (ESRD) may sign up for Medicare at any time. However, the date on which their Medicare coverage begins is usually on the fourth month after dialysis treatments begin but may be earlier if certain conditions are met.

18. Parts A and B After the Initial Enrollment Period

- Individuals who do not enroll in Part B (or Part A if they have to buy it) when they are first eligible, can enroll during a **General Enrollment Period** each year from January 1 – March 31.
 - Prior to January 1, 2023, coverage begins on July 1 of the year they enroll.
 - On or after January 1, 2023, coverage begins the first day of the month following the month in which the beneficiary enrolls.
- Individuals who have group health plan coverage based on their current employment or the employment of a spouse may enroll in Part A (if they have to buy it) and/or Part B anytime while covered under the group health plan or during a **Special Enrollment Period** that occurs during the 8-month period immediately following the last month they have group coverage.
- Beginning January 1, 2023, the Secretary may establish special enrollment periods in the case of individuals who are entitled to Part A, have attained age 65, meet residency requirements and meet such exceptional conditions as the Secretary of HHS specifies.

19. Medicare Part A Entitlement and Part B Enrollment

An individual is entitled to Part A if he or she is eligible for premium-free Part A or if the individual has enrolled in Part A and continues to pay the premium (or have the premium paid on his/her behalf).

For an individual to enroll in Part B and remain enrolled in Part B, the individual must pay the Part B premium (or have the premium paid on his/her behalf).

20. Other Ways to get Medicare - Eligibility Overview

To get Medicare benefits other than through Original Medicare, beneficiaries must meet certain eligibility criteria.

Part C ¹	Part D	Cost Plans	Medicare-Medicaid Plans	PACE Plans
Individuals must: ○ be entitled to Part A <u>and</u> enrolled in Part B; and ○ reside in the MA plan's service area.	Individuals must: ○ be entitled to Part A <u>and/or</u> enrolled in Part B; and ○ reside in the Part D plan's service area.	Individuals must: ○ be entitled to Part A <u>and/or</u> enrolled in Part B (if they are not entitled to Part A, they will not have coverage of Part A benefits under the cost plan); and ○ reside in the cost plan's service area.	Individuals must: ○ be eligible for both Medicare and Medicaid; ○ meet eligibility requirements specific to the state; and ○ reside in the plan's service area.	Individuals must: ○ Be age 55 or older; ○ be certified as eligible for nursing home care by their state; ○ be able to live safely in a community setting at the time of enrollment; ○ reside in the PACE organization's service area; ○ Meet any additional program-specific eligibility conditions imposed under the plan's PACE Program Agreement

¹ Note that certain types of Part C plans such as Medical Saving Account plans and Special Needs Plans have additional eligibility requirements.

21. Medicare Premiums—Part A

Most individuals are entitled to Part A without paying a premium.

- For individuals age 65 or older to be entitled to premium-free Part A, the individual or their spouse must have worked and paid Medicare taxes for at least 10 years; or
- All individuals eligible for Medicare due to a disability, End-Stage Renal Disease (ESRD), or Amyotrophic Lateral Sclerosis (ALS) are eligible for premium-free Part A.

For those individuals who do not automatically qualify for premium-free Part A coverage, the monthly Part A premium in 2022 is:

- \$499, for individuals or their spouses who paid Medicare taxes for less than 30 quarters.
- \$274, for individuals or their spouses who paid Medicare taxes for 30-39 quarters.
- Individuals who are not eligible for premium-free Part A and those who don't buy Part A when they are first eligible may pay a late penalty of up to 10% unless they enroll during a special enrollment period. (They will have to pay the higher premium for twice the number of years they could have had Part A but did not sign up.)

22. Medicare Premiums for Part B

Beneficiaries enrolled in Part B must pay a monthly premium.

- In 2022, the standard monthly premium for Part B is \$170.10. Most people pay the standard monthly premium. However, some people pay more based on their income (as reported to the IRS two years prior in 2020).

23. Medicare Premiums for Part B and the IRMAA

- Individuals with incomes in 2020 over \$91,000 or filing jointly with incomes over \$182,000, pay more in 2022, up to \$578.30 a month, based on the income-related monthly adjustment amount (IRMAA).

Individual tax return	Joint tax return	2021 Part B premium
< \$91,000	<\$182,000	\$170.10
>\$91,000 to \$114,000	>\$182,000 to \$228,000	\$238.10
>\$114,000 to \$142,000	>\$228,000 to \$284,000	\$340.20
>\$142,000 to \$170,000	>\$284,000 to \$340,000	\$442.30
>\$170,000 to \$500,000	>\$340,000 to \$750,000	\$544.30
= or > \$500,000	= or > \$750,000	\$578.30

* There are separate standards for beneficiaries who are married and lived with their spouses at any time during the year, but who file separate tax returns from their spouses

24. Medicare Premiums for Part B: Payment Mechanisms and Penalties

- Part B premiums may be deducted from Social Security checks, Railroad Retirement checks, or Office of Personnel Management (civil service annuity) checks. If an individual does not get these checks, they will get a premium bill from Medicare every 3 months.
- Employers may pay monthly Part B premiums on behalf of retirees.
- For individuals who do not enroll in Part B when first eligible, the Part B premium is increased 10% for each full 12-month period the beneficiary could have had Part B but did not enroll. This is known as a “late enrollment penalty.”

Exception from Penalty: Individuals who have group health plan coverage based on their current employment or the employment of a spouse are not subject to the premium increase penalty if they enroll in Part B anytime while covered under the group health plan or during the special enrollment period that occurs during the 8-month period immediately following the last month they have group coverage.

25. Medicare Premiums for Part B – Examples

Example: Ms. Stein retires from her job when she is 69. She did not previously enroll in Part B because she had employer group coverage. Her last month of group coverage is in February. She has until October to enroll in Medicare Part B without incurring a late enrollment penalty.

Example: Mr. O'Hare, who is 70 and does not have Part B, is retired, but he has health coverage through his wife's current employer. If Mr. O'Hare decides to get Part B while the group coverage is in effect or within 8 months after his last month of group coverage, he can do so without incurring a late enrollment penalty.

26. Title Page - Help for Individuals with Limited Income

27. Help for Individuals with Limited Income/Resources

- Beneficiaries may qualify for help to pay the Medicare Part A (if any) and Part B premium, the Part A and Part B deductibles and cost-sharing, and/or some Part D prescription drug costs.
- Beneficiaries may qualify for the following programs by applying to the State Medicaid office:
 - Medicare Savings Program: help paying for the Medicare Part A and Part B premiums and, in some cases, deductibles and cost-sharing.
 - The "Qualified Medicare Beneficiary" program is one type of Medicare Savings Program. Qualified Medicare Beneficiaries enrolled in Medicare Advantage plans also get help with their Medicare Advantage cost-sharing amounts.
 - Part D low-income subsidy (also known as "Extra Help"): help to pay for prescription drug coverage. Persons interested in Part D help only may also call the Social Security Administration (SSA) at 1-800-772-1213 or apply online at www.ssa.gov/prescriptionhelp. Extra help isn't available in Puerto Rico, the U.S. Virgin Islands, Guam, the Northern Mariana Islands, or American Samoa.
 - Persons who do not qualify for the Part D low-income subsidy but are of limited means may qualify for help in paying Part D drug costs through a State's Pharmaceutical Assistance Program.
 - Medicaid: help with health care costs not covered by Medicare, such as custodial/long term care.

28. Assisting Individuals with Limited Income/Resources

- Beneficiaries with limited income and resources should be encouraged to apply to their State Medicaid office to determine eligibility for various (Federal or State) programs.
- Agents should tell beneficiaries who may be eligible to call or visit their Medicaid office and ask for information on Medicare Savings Programs. To get the phone number for the state, visit Medicare.gov/contacts or call 1-800-MEDICARE (1-800-633-4227) or contact the State Health Insurance Assistance Program (SHIP).

29. Title Page - Medicare Part A Benefits and Original Medicare Cost Sharing

30. Medicare Part A Benefits

- Part A provides coverage for:
 - Inpatient hospital care (including care provided by acute care hospitals, critical access hospitals, inpatient rehabilitation facilities, and long-term care hospitals)
 - Skilled nursing and rehabilitation care, but only after a three-day hospital stay (Medicare Advantage plans may waive the 3-day stay requirement)
 - Blood
 - Hospice care
 - Up to 100 days of home health care after an individual is in a hospital or skilled nursing facility (SNF)
 - Inpatient psychiatric care (up to 190 lifetime days)

31. Medicare Part A – Original Medicare Cost-Sharing for Inpatient Hospital Care

In 2022, beneficiaries pay the following amounts for inpatient hospital care covered under Original Medicare:

- \$1,556 deductible for each benefit period
 - A benefit period begins the day an individual is admitted to a hospital or skilled nursing facility (SNF) and ends when an individual has not received hospital or SNF care for 60 days in a row.
- Days 1–60: \$0 coinsurance for each benefit period
- Days 61–90: \$389 coinsurance per day of each benefit period
- Days 91 and beyond: \$778 coinsurance per each "lifetime reserve day" after day 90 for each benefit period
 - Lifetime reserve days are days a beneficiary may use after they have been in an inpatient hospital for 90 days. A beneficiary has 60 such days to use in their lifetime.
- Beyond lifetime reserve days: all costs

32. Medicare Part A – Original Medicare Cost-Sharing for Skilled Nursing and Rehabilitative Care

In 2022 beneficiaries pay the following amounts for skilled nursing and rehabilitative care covered under Original Medicare:

- Days 1-20: \$0 for each benefit period (as defined by Medicare)
- Days 21-100: \$194.50 coinsurance per day of each benefit period
- Days 101 and beyond: all costs

33. Title Page - Medicare Part B

34. Medicare Part B Benefits

Part B generally covers:

- Physician and other health care professional services
- Outpatient hospital services
- Clinical lab and diagnostic tests, such as X-rays, MRIs, CT scans
- Medical equipment
- Home health care is not covered under Part A (because the individual was not in a hospital or SNF or has exceeded 100 days)
- Physical therapy
- Bariatric surgery for individuals who meet certain conditions related to morbid obesity
- Chemotherapy provided on an outpatient basis

35. Other Part B Items and Services

- Ambulance services
- Chiropractic services – for limited situations
- Opioid use disorder treatment
- E-visits
- Diabetic supplies
- Vaccines
- Kidney dialysis
- Outpatient mental health care (limits apply)
- Certain telehealth services
- Transplant physician services and drugs

36. Medicare Part B – Original Medicare Cost Sharing

In 2022, beneficiaries pay the following amounts for Part B services covered under Original Medicare:

- A \$233 annual deductible. The deductible does not apply to certain Part B covered preventive services.
- After the deductible is satisfied, beneficiaries typically pay 20% of the Medicare-approved cost for Part B covered services.

37. Medicare Part B Benefits - Preventive Services and Screenings

Beneficiaries covered under Original Medicare and Medicare Advantage plans will have no cost-sharing for most preventive services. Preventive services include, but are not limited to:

- One-time “Welcome to Medicare” physical
- Annual wellness visit after 12 months enrolled in Part B and annually thereafter
- Immunizations – pneumococcal, hepatitis B, annual flu shot (Note: shingles shots are covered under Part D, not Part B)
- Bone mass measurement – every 24 months for certain conditions or meets certain criteria
- Obesity behavior therapy
- Pap test and pelvic examination - every 24 months for all women; every 12 months for those at high risk
- Diabetes self-management training – for persons with diabetes
- Smoking and tobacco-use cessation counseling – for any illness related to tobacco use
- Glaucoma testing – once per year for those at high risk

Screenings include, but are not limited to:

- Mammogram (Breast Cancer Screening) – annual screening for most women
- Depression Screening – every 12 months
- Colorectal cancer screening
- Diabetes screenings – up to two per year for those with risk factors
- Prostate cancer screening – every 12 months for men over age 50

38. Not Covered by Medicare Part A & B

- Routine dental care/dentures
- Cosmetic surgery
- Custodial/long term care
- Health care while traveling outside the US
- Hearing aids
- Outpatient prescription drugs (this is covered under Part D)
- Massage Therapy
- Routine eye care and eyeglasses
- Concierge care (also called concierge medicine, retainer-based medicine, boutique medicine, platinum practice, or direct care)
- Covered items or services provided by a doctor or other provider who has opted out of Medicare (except in the case of an emergency or urgent need)

39. Title Page - Original Medicare

40. Original Medicare and Part D Prescription Drug Coverage

- A beneficiary in Original Medicare may receive Part D prescription drug coverage through a stand-alone prescription drug plan (PDP).
- Generally, except for those dually eligible for Medicare and Medicaid, Medicare beneficiaries must actively select a Part D plan.
- Beneficiaries who enroll in Part D typically pay a monthly premium, annual deductible, and per-prescription cost-sharing.
- In selecting a Part D plan, beneficiaries should consider expected premiums and cost sharing, formulary, and network pharmacies among other factors.

41. Appeals related to Original Medicare Part A and Part B Coverage and Payment Determinations

Beneficiaries receiving their Part A and/or Part B services through Original Medicare have a right to appeal Medicare coverage and payment decisions.

- Beneficiaries should look at their "Medicare Summary Notice" (MSN).
- The MSN shows the Part A and Part B services and supplies that providers and suppliers billed to Medicare on their behalf.
- The MSN also shows what Medicare paid on the beneficiary's behalf and what the beneficiary may owe the provider. The MSN also shows if Medicare has fully or partially denied their medical claim.
- Beneficiaries can also track their Medicare claims or view electronic MSNs by visiting MyMedicare.gov.
- Beneficiaries must file an appeal related to Part A or B services within 120 days of the date they get the MSN in the mail. The appeal should be sent to the Medicare Administrative Contractor (MAC) that processed their claim (indicated on the MSN). Instructions for filing an appeal can be found on Medicare.gov.
- If a beneficiary disagrees with the MAC's decision on the appeal, they have 180 days after getting the decision notice to request a reconsideration by a Qualified Independent Contractor (QIC).
- Additional levels of appeal may also be available, depending on the amount in controversy.

42. Fast appeals under Original Medicare for Certain Services

- Beneficiaries receiving their Part A and/or Part B services through Original Medicare have a right to a fast appeal if they believe certain Medicare-covered services are ending too soon.
- This includes services provided by a hospital, skilled nursing facility, home health agency, comprehensive outpatient rehabilitation facility, or hospice.
- Their provider will give them written notice before the end of their services. The notice tells them how to ask for a fast appeal.

43. Grievances under Original Medicare

Beneficiaries may also file complaints about their Medicare providers or the quality of care they received. For example, a beneficiary may have a complaint about:

- Unprofessional conduct by your provider
- improper care
- unsafe conditions

Instructions for filing grievances can be found at: <https://www.medicare.gov/claims-appeals/how-to-file-a-complaint-grievance>

44. Additional Beneficiary Protections under Original Medicare

- Medicare operates a 24-hour helpline at 1-800-Medicare. (TTY users should call **1-877-486-2048**.)
 - Beneficiaries can use this number to find out about their claim status, coverage and benefits, premium payments, or to ask other questions about Medicare.
- Beneficiaries can also get assistance with Medicare, including help filing an appeal or grievance, through their local State Health Insurance Assistance Program (SHIP).
 - Contact information for their SHIP can be found at <https://www.shiptacenter.org/>

45. For More Information about Medicare

- Centers for Medicare & Medicaid Services (technical information) www.cms.gov
- Medicare (beneficiary audience) www.medicare.gov
- Medicare & You Handbook <https://www.medicare.gov/pubs/pdf/10050-Medicare-and-You.pdf>
- Your Medicare Benefits handbook <https://www.medicare.gov/Pubs/pdf/10116-your-medicare-benefits.pdf>

46. Title Page: Medigap Coverage

47. Medigap (Medicare Supplement Insurance)

Medigap insurance:

- Works only with Original Medicare.
- Is sold by private insurance companies to fill “gaps” in Original Medicare coverage, such as all or part of the deductibles or coinsurance amounts.
- It coordinates with Original Medicare coverage.
- Some Medigap policies cover limited benefits not covered by Part A or Part B of Original Medicare, such as extra days of coverage for inpatient hospital care or foreign travel emergency care. Generally, Medigap doesn’t cover long-term care (like care in a nursing home), vision or dental services, hearing aids, eyeglasses, or private-duty nursing.

48. Medigap is NOT

- Medigap is NOT a Medicare Advantage (Part C) plan or other Medicare health plan.
- Medigap is NOT original Medicare. Medigap supplements Original Medicare benefits only.
- In addition,
 - A Medigap plan cannot be used with a Medicare Advantage health plan.
 - It is illegal to sell a Medigap plan to someone already in a Medicare Advantage health plan.

49. Further Information on Medigap (Medicare Supplement Insurance)

- Medigap policies are available in standardized benefit plans, identified by certain letters between A and N (however, different plans are offered in Massachusetts, Minnesota, and Wisconsin).
- Turning age 65 and signing up for Part B triggers a six-month Medigap open enrollment period when Medigap insurers must issue you a policy, regardless of any pre-existing conditions. This is called a guaranteed issue right.
- In certain limited instances, leaving a Medicare Advantage plan may trigger a guaranteed issue right. Some states have additional guaranteed issue periods for Medicare beneficiaries. Agents should look into state-specific Medigap laws.

50. Medigap Coverage

All Medigap plans pay for some or all of the following costs:

- Part A coinsurance
- Coverage for 365 additional hospital days when Medicare coverage for hospitalization ends
- Part B coinsurance or copayment
- Blood (First 3 pints)
- Hospice care coinsurance or copayment

51. Beneficiaries with Medigap Plans with/without Drug Coverage

- Medigap plans H, I, and J offer non-Medicare drug coverage. These plans could no longer be sold as of January 1, 2006. However, some beneficiaries may have decided to keep their H, I, or J policy with the drug coverage they had before January 1, 2006.
- Individuals who are enrolled in Medigap plans may only obtain Medicare drug coverage (Part D) through a stand-alone prescription drug plan.
- To enroll in Part D, individuals who have Medigap plans H, I or J may:
 - keep their Medigap coverage with the drug portion of the coverage removed and enroll in a Part D PDP plan; or
 - drop their Medigap coverage and enroll in an MA-PD or other health plans with a PDP.

52. Beneficiaries with Medigap Drug Coverage – Creditable Coverage

- Non-Medicare insurers (including Medigap plans) are required to notify beneficiaries annually whether or not the prescription drug coverage they have is creditable (coverage that expects to pay, on average, at least as much as Medicare's standard Part D coverage expects to pay).
- All beneficiaries who do not maintain creditable coverage must pay a Part D late enrollment penalty if they wish to enroll in Part D unless they qualify for 'Extra Help' or enroll in Part D during the special enrollment period for loss of creditable coverage (discussed later).
- Beneficiaries who previously had creditable coverage and are informed that their non-Medicare drug coverage is no longer creditable will have a special enrollment period to enroll in a Part D plan without the obligation to pay a Part D late enrollment penalty.

53. Changes in Medigap for individuals who become eligible for Medicare after December 31, 2019

- Newly eligible individuals may not purchase a Medigap plan that pays the Part B deductible (generally plans C, F or high deductible F, but the prohibition also applies in waiver states with non-standard packages).
 - "Newly eligible individuals" are defined as those: (1) who have attained age 65 on or after January 1, 2020; or (2) first become eligible for Medicare due to age, disability or end-stage renal disease on or after January 1, 2020.
- Individuals enrolled in plans that cover the Part B deductible may remain enrolled in those plans.
- Individuals who became eligible for Medicare before 2020 may enroll in plans that cover the Part B deductible.

54. Medigap Plans

Plans available to all beneficiaries									Plans available only to beneficiaries eligible before 2020	
Medigap Benefits	A	B	D	G ⁵	K ⁴	L ⁴	M	N	C	F ¹
Part A Coinsurance and Hospital Benefits	X ²	X	X	X	X	X	X	X	X	X
Part B Coinsurance or Copayment	X	X	X	X	50%	75%	X	X ³	X	X
Blood (First 3 pints)	X	X	X	X	50%	75%	X	X	X	X
Part A Hospice Care Coinsurance/ Copayment	X	X	X	X	50%	75%	X	X	X	X
Skilled Nursing Facility Care Coinsurance			X	X	50%	75%	X	X	X	X

1. Plan F also offers a high-deductible plan that is only available to individuals eligible for Medicare before January 1, 2020. In 2022, a policyholder pays \$2,490 before the Medigap policy pays anything.

2. "X" indicates that coverage is 100% of the Medicare allowable amount. A percentage number indicates the proportion of the Medicare allowable amount covered.

3. Plan N has a copayment of up to \$20 for physician office visits and up to \$50 for emergency room visits (waived in certain circumstances).

4. Plans K and L pay 100% after out-of-pocket limit is reached. In 2022 the out-of-pocket limits for Plan K and Plan L are \$6,620 and \$3,310, respectively.

5. There is a high deductible version of Plan G. The deductible for 2022 is \$2,490.

55. Medigap Plans, continued

Plans available to all beneficiaries									Plans available only to beneficiaries eligible before 2020	
Medigap Benefits	A	B	D	G ⁵	K ⁴	L ⁴	M	N	C	F ¹
Medicare Part A Deductible		X	X	X	50%	75%	50%	X	X	X
Medicare Part B Deductible									X	X
Medicare Part B Excess Charges				X						X
Foreign Travel Emergency (up to plan limits) ³			80%	80%			80%	80%	80%	80%

1. Plan F also has a high-deductible option. In 2022, a policyholder pays \$2,490 before the Medigap policy pays anything. Once the plan deductible is met, the plan pays 100% of covered services for the rest of the calendar year. However, high deductible Plan F counts payment of the Medicare Part B deductible toward meeting the plan deductible.

2. "X" indicates that coverage is up to 100% of the Medicare allowable amount. A percentage number indicates the proportion of the Medicare allowable amount covered, except for foreign travel.

3. The foreign travel benefit pays 80% of charges after a \$250 deductible, up to a \$50,000 lifetime maximum.

4. Plans K and L pay 100% after out-of-pocket limit is reached. In 2022 the out-of-pocket limits for Plan K and Plan L are \$6,620 and \$3,310, respectively.

5. There is a high deductible version of Plan G. The deductible for 2022 is \$2,490.

56. Medigap Plans – Case Study

Ms. Smith wishes to buy a Medigap plan. She explained that she wishes to get a plan with a lower premium and doesn't mind paying more when she uses services, up to a limit. The agent correctly directed her to choose between Plans K and L to meet her needs.

57. For More Information about Medigap

- Centers for Medicare & Medicaid Services:
<http://www.cms.gov/Medigap/>
- 2021 Medicare & You Handbook:
https://www.medicare.gov/sites/default/files/2020-12/10050-Medicare-and-You_0.pdf